

MedReach Pharmaceuticals — Executive Summary

Business Context

MedReach Pharmaceuticals expanded its field force by 20% last year to accelerate prescription growth across six Indian zones. Despite this increase in headcount, overall sales growth remained stagnant. To identify the underlying causes, we analysed 1,208 MR activity records covering 200 Medical Representatives and 500 doctors across multiple product portfolios and geographies.

Key Findings

1. Significant Conversion Gap Between New and Existing MRs

New hire MRs convert only 6% of calls into prescriptions, compared to 46% for existing MRs, an approximately **8x** performance gap. This disparity cannot be explained by experience alone and strongly suggests **systemic onboarding** and **field training failures** rather than **isolated individual underperformance**.

Additionally, new hires averaged 13.9 calls per month compared to 19.9 for existing MRs, resulting in approximately **2,880 lost annual call opportunities** across the current new-hire cohort.

2. Low-Value Doctors Are Being Prioritised Over High-Potential Segments

Doctor segmentation analysis revealed that Segment A doctors generate an average of 147.7 prescriptions per month, while Segment D doctors generate only 11.6. This means a single Segment A doctor carries **approximately 12.7x** the prescription potential of a Segment D doctor.

Despite this, a disproportionate amount of MR effort is currently allocated toward lower-yield Segment D doctors, indicating that call planning is being driven by activity targets rather than prescription opportunity.

3. New Hire Deployment Strategy Is Misaligned

22.5% of new hires were deployed in the Central zone, which already had strong existing MR coverage with a 45% conversion rate. Meanwhile, underpenetrated zones such as North and West received comparatively fewer resources.

This suggests that deployment decisions were based on administrative allocation rather than territory opportunity and market penetration potential.

Recommendations

1. Implement Structured New-Hire Onboarding

Introduce a **mandatory 90-day onboarding and coaching programme** for all new MRs, including weekly field mentoring by Territory Managers, product-positioning workshops, and doctor engagement simulations.

The programme should target a **minimum 15% conversion rate** within the **first three months** of deployment.

If current new hires were to perform at the same conversion rate as existing MRs, MedReach could generate approximately **220 additional prescriptions per month** without increasing headcount.

2. Reallocate MR Effort Toward High-Potential Doctors

Rebalance at least **20%** of current Segment D doctor coverage toward Segment A doctors with significantly higher prescription potential.

Simultaneously, revise MR KPIs from activity-based metrics (calls made) toward outcome-based metrics (prescriptions generated) to incentivise quality of engagement over quantity of visits.

Based on current prescription differentials, this reallocation could generate approximately **531 additional prescriptions per month**.

3. Optimise Territory Deployment Strategy

Conduct quarterly territory allocation reviews to ensure that new hires are deployed into underpenetrated and growth-oriented zones rather than duplicating already-successful coverage.

Future hiring and deployment decisions should incorporate territory-level conversion rates, doctor density, and prescription opportunity scoring.

Quantified Business Opportunity

Implementing these recommendations could unlock an estimated **₹45 Lakhs in additional annual prescription revenue**, equivalent to a 12x return on the cost of a structured onboarding programme without any incremental headcount investment.

More importantly, the analysis suggests that MedReach's growth constraint is not workforce size, but workforce productivity, targeting strategy, and deployment efficiency.

Addressing these structural issues will allow the organisation to improve sales output using its existing field force before considering further expansion.